



Veterans Administration

VHA FAX TRANSMITTAL

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**Audie L. Murphy VA Hospital,
STVHCS**

7400 Merton Minter
San Antonio, TX 78229
210-617-5300

DATE:	Apr. 23, 2024
ATTN:	
FROM:	Community Care
SUBJECT:	Referral

Note:

Good afternoon,
Please see referral for review and scheduling

Thank you



U.S. Department
of Veterans Affairs

**VA Form 10-7080 - Approved Referral For
Medical Care**

Veteran Name: Davis, John Nelson

Veteran ICN: 1028225168V143920

Veteran EDIPI: 1188219248

Veteran Date of Birth: 1973-09-09

Veteran Preferred Name:

Pronoun:

Pronoun Description:

Veteran Address: 407 W GOODWIN AVE
VICTORIA, TX 77901

Veteran Phone Number: (315)486-9807

Veteran Mobile Phone Number (if Known): (315)486-9807

Veteran Business Phone Number (If Known): (315)486-5313

Veteran Email Address (If Known): JOHNNDAVIS09@GMAIL.COM

Referral Number: VA0036825963

Priority: Routine

Referral Issue Date: 2024-04-01

Expiration Date: PRELIMINARY 2024-09-28 (SEE BELOW)*

First Appointment Date: SUPPLY TO VA ASAP

Referring VA Facility: Audie L. Murphy Memorial Veterans' Hospital

VA Telephone Number: 210-949-3850

VA Fax Number:

Initial Community Care Provider/Facility: VICTORIA DERMATOLOGY PA

Initial Provider Location: VICTORIA DERMATOLOGY PA-115 Medical Dr ; Ste 202, Victoria, TX, 77904-207N00000X

Provider Name (if known): PATEL, AMIT

Provider Phone: 281-836-4690

Provider Fax: 409-729-2449

Community Provider NPI: 1346553120

Caregiver Type: Veteran does not rely on a caregiver

Caregiver Details:

***IN ORDER TO PROVIDE THE FULL DURATION OF THE STANDARDIZED EPISODE OF CARE (SEOC)
PLEASE CONTACT THE REFERRING VA FACILITY WITH A FIRST APPOINTMENT TO CALCULATE
A FINAL EXPIRATION DATE. CLAIM PAYMENTS ARE DEPENDENT UPON THESE DATES.**

Any claim related to this episode of care **MUST INCLUDE THE APPROVED REFERRAL NUMBER** as the
Referral Number or Prior Authorization number.

**Please see below for Additional VA Referring Facility Information, Billing Information
and Appointment/Provider Information.**

Pertinent Clinical Information

**Please view the Clinical Information in the VA Order section for more information related to the Original VA Order
Reason for Request.**

Chief Complaint: Soft Tissue Disorder, unspecified(ICD-10-CM M79.9) PT WITH H/O SKIN CANCER WITH LARGE
TUMOR ON HIS BACK REMOVED 2005 AND RECURRENCE 2011

Patient History / Clinical Findings / Diagnosis (Co-Morbidities): oft Tissue Disorder, unspecified(ICD-10-CM M79.9)
PT WITH H/O SKIN CANCER WITH LARGE TUMOR ON HIS BACK REMOVED 2005 AND RECURRENCE 2011 Is this

referral for Service Connected condition? No Per VA PBM Formulary Process Management Policy from VHA Handbook 1108.08, the use of drugs solely to improve normal physiologic function or to enhance body appearance is generally not considered medically necessary and therefore are not to be prescribed. Medications used for this purpose are considered cosmetic. The use of medications for hair re-growth (e.g., minoxidil, finasteride, or pimecrolimus) are considered cosmetic in nature and should not be prescribed unless care is being provided in connection with the treatment of a service-connected injury.

Provisional Diagnosis: M799 Soft tissue disorder, unspecified

Services Authorized

The VA Order Reason for Request is the official clinical order. This scope of services associated with the medical care for this authorization is found below. Necessary services that are not included must be requested using the Request for Services procedures. Please visit the VHA Storefront www.va.gov/COMMUNITYCARE/providers/index.asp for additional resources and requirements.

Service Requested: Dermatology_PRCT SEOC 1.0.12

Category of Care: DERMATOLOGY

Procedural Overview - Standardized Episode of Care (SEOC)

Dermatology_PRCT SEOC 1.0.12 Duration: 180 Days

Description:

This authorization covers services associated with the specialty(s) identified for this episode of care, including all medical care listed below relevant to the referred care specified on the consult/referral order.

NOTE: Please note this SEOC does not cover cosmetic procedures that are not considered medically necessary or correct a functional disability as they are excluded from the Medical Benefits Package.

NOTE: This SEOC does not cover Mohs. A separate referral specifically for Mohs with the Mohs SEOC is required.

NOTE: Superficial Radiation Therapy (SRT) is not covered in this episode of care. SRT is only to be performed and approved by a radiation oncologist and not a dermatologist. SRT must be requested through an RFS and approved using one of the radiation therapy SEOCs.

NOTE: Additional consultations needed relevant to the patient complaint/condition require VA review and approval including a referral to another specialty for secondary/complex closure.

NOTE: This SEOC does not cover the removal of skin tags, seborrheic keratosis, and other benign skin lesions unless they are bleeding, painful, or changing.

No.	Service/Procedure	Number Of Visits Authorized
1	Initial outpatient evaluation (including full body exam), treatment, and follow-up visits for the referred condition on the consult/referral order	999
2	Diagnostic imaging relevant to the referred condition on the consult/referral order	999

3	Labs and pathology relevant to the referred condition on the consult/referral order	999
4	Procedures performed by the dermatology provider relevant to the referred condition on the consult/referral order including but not limited to: cryotherapy, biopsy (e.g., excisional, shave, punch), excision, repair, excimer laser, photodynamic therapy, UVB therapy, debridement, medically necessary hair removal	999

Additional Information:

- * Please visit the VHA Storefront www.va.gov/COMMUNITYCARE/providers/index.asp for additional resources and requirements pertaining to the following:
- * Pharmacy prescribing requirements
- * Durable Medical Equipment (DME), Prosthetics, and Orthotics prescribing requirements
- * Precertification (PRCT) process requirements
- * Request for Services (RFS) requirements

REFER ALL QUESTIONS RELATED TO THIS APPROVAL TO THE ISSUING VA OFFICE**Referring VA Facility:** Audie L. Murphy Memorial Veterans' Hospital**Station Number:** 671**Telephone Number:** 210-949-3850**Address:** 7400 Merton Minter Boulevard SAN ANTONIO TX 78229**Referring Provider:** Zaiontz, Theresa**Referring Provider NPI:** 1043508344**Unique Consult No:** 671_8536528**Program Authority:** Authorized/Pre-authorized VA Referral (not otherwise specified) - 1703**Affiliation:** Triwest**Network:** CC Network 4**Appointments/Providers Assigned to the Referral**

Provider/Facility Name	Provider/Facility Location	Appt Date	Appt Time	Telephone #	Fax #
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Additional Service Information**Request for Services (RFS)**

A Request for Services (RFS) is a provider-generated request for new or additional care outside the scope of the current approved referral/authorization. Provider should always submit the RFS directly to the authorizing VAMC, preferably via the HSRM portal. Providers should always submit an RFS on the same day it is determined it's needed and before delivering care, unless it is emergent care. In that case, the RFS can be submitted simultaneously.

How to Submit an RFS

VA prefers providers submit an RFS via the HSRM portal, available on the VA's website

- Go to the VA Storefront at <https://www.va.gov/COMMUNITYCARE/providers/Care-Coordination.asp#RFS>
 - Navigate to the link to the RFS form at the bottom of the section

Medical Records and Documents Requirements

Medical Records and documentation are required for all provider services. Providers are required to submit Medical documentation directly to the authorizing VAMC, preferably via upload to HSRM.

Billing and Other Referral Information**Submitting Claims**

ANY CLAIMS RELATED TO THIS EPISODE OF CARE MUST BE SUBMITTED TO TRIWEST OR DELTA DENTAL AS APPROPRIATE AND INCLUDE THE APPROVED REFERRAL NUMBER.**Methods to submit claims:****Electronic Data Interchange (EDI):**

Payer ID for Medical – TWVACCN

Payer ID for Dental – CDCA1

More information on how to submit claims can be found by visiting

<https://www.va.gov/COMMUNITYCARE/revenue-ops/Veteran-Care-Claims.asp>**Precertification**

The Standardized Episode of Care (SEOC) referral you have accepted does not include services that require third-party payer (TPP) precertification.

Community Care Medical Policies

VA Community Care Medical Policies describe standard VA health care benefits for services and procedures that community providers may recommend as necessary for a Veteran. Prior to providing care, providers should use the Community Care Medical Policies as a reference when determining if a Veteran meets VA clinical criteria. When additional services are requested, Community Care Medical Policies will be used to determine approval by a clinical reviewer. Community Care Medical Policies and supporting information can be found at:

<https://www.va.gov/COMMUNITYCARE/providers/info-CDI.asp>**Medical Devices**

As part of furnishing the services authorized on the approved referral, you may recommend that medical devices, adaptive equipment, or other items be provided for the treatment or rehabilitation of the Veteran's medical condition. In such instances, you must complete a Request for Services (RFS) as outlined above.

<https://www.va.gov/COMMUNITYCARE/providers/Care-Coordination.asp#RFS>

The RFS must (1) indicate that the condition for which the item is being prescribed is within the scope of the authorized services on the approved referral, (2) describe the item or service being prescribed with as much specificity as possible (including manufacturer and model, needed custom measurements, size), and (3) provide a brief but thorough plain-language explanation of how the item or service will serve the treatment or rehabilitation needs of the Veteran. All parts of the RFS form must be filled out completely.

VA staff will determine whether the prescribed item or service is one that VA is authorized to purchase. If the item is not something that we are authorized to purchase, VA staff will work with you to identify an alternative that will best meet the Veteran's clinical needs. In making this determination, VA staff will consider your clinical judgment and expertise and work with you to resolve outstanding issues quickly.

In the event of an urgent or emergent need, you may provide medical devices, necessary items or services prescribed to Veterans receiving care in the community for urgent or emergent conditions at the time of healthcare service delivery or soon thereafter. Urgent or emergent DME or Medical Devices may include, but are not limited to splints, crutches, canes, slings, soft collars, walkers, and manual wheelchairs. DME dispensed directly to the Veteran based on an urgent or emergent need will be paid by the CCN Third Party Administrator. Claims for these items should be submitted using the process outlined in the **Submitting Claims** section of this form.

Pharmacy**Drug Safety and Administration Requirements**

· Must follow the VA National Protocol and clinical guidance for Esketamine or Ketamine administration. Ketamine treatments for mental health or for pain are not approved under a CCN referral. Prior to administration it is required the ordering and administering providers review the VA Protocol and Clinical Guidance found through the VA Formulary Search Tool available here:

· <https://www.pbm.va.gov/apps/VANationalFormulary/>

Must follow the VA Opioid Safety Guidelines and complete the required Opioid Safety Training found here:

- <https://www.pbm.va.gov/apps/VANationalFormulary/>
- <https://www.va.gov/COMMUNITYCARE/providers/EDU-Training.asp>

CVS Caremark is the retail pharmacy network for Veterans' immediately needed or Urgent/Emergent prescriptions.

Immediate need prescriptions:

- Must follow the VA Urgent/Emergent Formulary which can be found at <http://www.pbm.va.gov/PBM/nationalformulary.asp>
- Prescription can only go up to a 14-day supply. No refills of the immediate need medication may be authorized.
- Only a seven-day supply for opioids, or up to the opioid prescribing limit allowed by State—whichever is less—may be authorized.

Immediate need prescription extending past 14 days:

- The provider will need to send second prescription (beyond 14 days) to the referring VA medical facility's pharmacy for prescription fulfillment services.

Routine/maintenance prescriptions:

- Must be sent to the referring VA medical facility's pharmacy

If you do not have the ability to electronically submit prescriptions to pharmacies, please contact the Community Care representative at the referring VA medical facility for their pharmacy fax number. Please refer to:

<https://www.va.gov/COMMUNITYCARE/providers/Service-Requirements.asp> for additional instructions related to prescriptions.

Clinical Information on the VA Order

Reason for Request:

Provider to receive results: VOP PACT NP Orange

Type of Service: Evaluation and Treatment

Chief Complaint: Soft Tissue Disorder, unspecified (ICD-10-CM M79.9)

PT WITH H/O SKIN CANCER WITH LARGE TUMOR ON HIS BACK REMOVED 2005 AND RECURRENCE 2011

Patient History / Clinical Findings / Diagnosis (Co-Morbidities):

oft Tissue Disorder, unspecified (ICD-10-CM M79.9)

PT WITH H/O SKIN CANCER WITH LARGE TUMOR ON HIS BACK REMOVED 2005 AND RECURRENCE 2011

Is this referral for Service Connected condition? No

Per VA PBM Formulary Process Management Policy from VHA Handbook 1108.08, the

use of drugs solely to improve normal physiologic function or to enhance body

appearance is generally not considered medically necessary and therefore are not

to be prescribed. Medications used for this purpose are considered cosmetic.

The use of medications for hair re-growth (e.g., minoxidil, finasteride, or

pimecrolimus) are considered cosmetic in nature and should not be prescribed

unless care is being provided in connection with the treatment of a service-

connected injury.

**This 10-7080 - Approved Referral For Medical Care was generated on 04/23/2024 changes made to the referral after this date are not reflected on the form.



U.S. Department of Veterans Affairs

Veterans Health Administration
Office of Community Care

Health Share Referral Manager (HSRM): VA's Secure Online Portal for Managing Referrals and Authorizations

Why are community providers across the country excited to use HSRM?

- ◆ **Facilitates Health Information Exchange (HIE)** between community providers and VA via a unified platform
- ◆ Conveniently **organizes all active referrals** in one centralized location
- ◆ **Reduces time spent** waiting for fax, phone or email contact
- ◆ Allows community providers to **request authorization** for additional services or additional time to provide services
- ◆ Promotes **reduced turnaround time** for authorizations and reimbursement

HSRM Account Creation For Community Providers

- ◆ **STEP 1: Training:** Each staff member attends virtual training, completes eLearning lessons, or reviews one of the training guides
- ◆ **STEP 2: ID.me:** Each staff member creates an ID.me account and verifies their identity at <https://www.id.me/>
- ◆ **STEP 3: Submit EUT:** One facility point of contact (POC) fills out the End User Tracker (EUT), then sends it to HSRMSupport@va.gov
- ◆ **STEP 4: Receive Accounts:** Help Desk creates accounts in HSRM, then provides confirmation of account creation to the facility POC
- ◆ **STEP 5: Log Into HSRM:** Each staff member logs into HSRM at <https://ccracommunity.va.gov>
- ◆ Once these steps are complete, contact the VA Medical Center(s) you work with to let them know you have access to HSRM and to discuss your transition to HSRM

HSRM Registration Resources

- ◆ HSRM Support Points of Contact: <https://www.va.gov/COMMUNITYCARE/providers/HSRM-POC-List.asp>
- ◆ ID.me Registration Instructions: https://www.va.gov/COMMUNITYCARE/docs/pubfiles/factsheets/FactSheet_26-06.pdf#
- ◆ End User Tracker
 - End User Tracker Template: https://www.va.gov/COMMUNITYCARE/docs/providers/HSRM_End_User_Tracker.xlsx#
 - Send completed End User Tracker to HSRMSupport@va.gov
- ◆ HSRM Login: <https://ccracommunity.va.gov>

HSRM Educational Resources

- ◆ Live Virtual Training: https://www.train.org/virginia/course/1082953/live_event; please visit link to find available dates and to register in advance
- ◆ HSRM eLearning lessons: Log into <https://www.train.org/vha/welcome>, then search the course catalogue for "HealthShare Referral Manager"
- ◆ HSRM User Guide: https://www.va.gov/COMMUNITYCARE/docs/providers/HSRM_User_Guide.pdf
- ◆ HSRM Quick Reference Guide (QRG): https://www.va.gov/COMMUNITYCARE/docs/providers/HSRM_Quick_Reference_Guide.pdf#
- ◆ Additional information about HSRM: https://www.va.gov/COMMUNITYCARE/providers/Care_Coordination.asp
- ◆ HSRM Help Desk
Phone: 1-844-293-2272, Email: HSRMSupport@va.gov, Hours: Monday - Friday 7:30 a.m. - 5 p.m. Eastern Time

**Veterans Health
Administration****VHA Medical
Documentation**

Document Created: 04/23/2024 16:27

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AUDIE L MURPHY MEMORIAL VAMC 7400 Merton Minter Blvd, San Antonio, Texas 78229-4404
VA Referrals phone: 210-949-3850 Fax:

Point of Contact (POC): MARTINEZ, GRETCHEN
POC Phone: 2109493850 Fax:

Patient Name: DAVIS,JOHN NELSON

DOB: 09/09/1973

407 W GOODWIN AVE
VICTORIA, TEXAS 77901Phone (residential): (315)486-9807
Phone (mobile): (315)486-9807Referral Type: Community Care
Network**Next of Kin contact information**

DAVIS,CYNTHIA

Address (Next Of Kin)
407 W GOODWIN AVE
VICTORIA, TEXAS 77901

Phone: (315)486-5313

CONSULTS

Current PC Provider: COLLIS,LAURIE C
Current PC Team: VICTORIA PURPLE TEAM *WH*
Current Pat. Status: Outpatient
Primary Eligibility: SERVICE CONNECTED 50% to 100%
UCID: 671_8536528
Patient Type:
OEF/OIF: YES

Order Information

To Service: COMMUNITY CARE-DERMATOLOGY
Attention:
From Service: VOP PACT NP ORANGE
Requesting Provider: YBARRA,THERESA O
Service is to be rendered on an OUTPATIENT basis
Place: Consultant's choice
Urgency: Routine
Clinically Ind. Date: 2024-04-15 00:00:00

DST ID:
 Orderable Item:
 Consult: Consult
 Provisional Diagnosis: Soft Tissue Disorder, Unspecified (ICD-10-CM M79.9)
 Reason For Request:
 Provider to receive results: VOP PACT NP Orange
 Type of Service: Evaluation and Treatment

Chief Complaint: Soft Tissue Disorder, unspecified (ICD-10-CM M79.9)
 PT WITH H/O SKIN CANCER WITH LARGE TUMOR ON HIS BACK REMOVED 2005 AND RECURRENCE 2011

Patient History / Clinical Findings / Diagnosis (Co-Morbidities):

Soft Tissue Disorder, unspecified (ICD-10-CM M79.9)
 PT WITH H/O SKIN CANCER WITH LARGE TUMOR ON HIS BACK REMOVED 2005 AND RECURRENCE 2011

Is this referral for Service Connected condition? No

Per VA PBM Formulary Process Management Policy from VHA Handbook 1108.08, the use of drugs solely to improve normal physiologic function or to enhance body appearance is generally not considered medically necessary and therefore are not to be prescribed. Medications used for this purpose are considered cosmetic. The use of medications for hair re-growth (e.g., minoxidil, finasteride, or pimecrolimus) are considered cosmetic in nature and should not be prescribed unless care is being provided in connection with the treatment of a service-connected injury.

Inter-facility Information

AUDIE L. MURPHY MEMORIAL HOSP

Status: ACTIVE

Last Action: RECEIVED

Facility

Activity	Date/Time/Zone	Responsible Person	Entered By

CPRS RELEASED ORDER	04/01/24 11:03	YBARRA,THERESA O	YBARRA,THERESA O
ADDED COMMENT	04/01/24 00:00	YBARRA,THERESA O	YBARRA,THERESA O
RECEIVED	04/01/24 15:03	LARS,SUSAN M	LARS,SUSAN M

Note: TIME ZONE is local if not indicated

No local TIU results or Medicine results available for this consult

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===== END =====

PROBLEM LIST

Sensitive Diagnoses

No sensitive diagnoses were provided.

Other Diagnoses

Problem	Code
Contact with and exposure to other hazardous subst	Z77.29
Dysphagia, unspecified	R13.10
Finger pain	R69.

Functional dyspepsia	K30.
Hyperglycemia, unspecified	R73.9
Low back pain, unspecified	M54.50
Male erectile disorder	F52.21
Malignant melanoma of other part of trunk	C43.59
Obesity, unspecified	E66.9
Obstructive sleep apnea (adult) (pediatric)	G47.33
Pain in unspecified foot	M79.673
Pain in unspecified hip	M25.559
Plantar fascial fibromatosis	M72.2
Post-traumatic stress disorder, chronic	F43.12
Prediabetes	R73.03
Shoulder joint pain	R69.
Tobacco use	Z72.0
Unilateral inguinal hernia, w/o obst or gangrene,	K40.91

MEDICATIONS

100 most recent outpatient medications released by VA to Veteran in the last 6 months

Medication Name and Dose	Quantity	Refill Number	Issue and Fill Date	Status
IBUPROFEN 800MG TAB	Qty:200	Fill: 2 of 3	Orig: 2023-10-03 Last: 2024-02-04	ACTIVE
TAKE ONE TABLET BY MOUTH THREE TIMES DAILY WITH MEALS AS NEEDED FOR PAIN TAKE WITH FOOD OR MILK.				
METFORMIN HCL 500MG TAB	Qty:180	Fill: 1 of 2	Orig: 2024-04-01 Last: 2024-04-01	ACTIVE
TAKE ONE TABLET BY MOUTH TWICE A DAY FOR DIABETES PREVENTION WITH MEALS				

ALLERGIES

No documented allergies

PROGRESS NOTES

LOCAL TITLE: VOPC PROVIDER NOTE

STANDARD TITLE: PRIMARY CARE NOTE

DATE OF NOTE: APR 01, 2024@09:26 ENTRY DATE: APR 01, 2024@09:26:49

AUTHOR: YBARRA,THERESA O EXP COSIGNER:

URGENCY: STATUS: COMPLETED

DATE: APR 01, 2024

DAVIS,JOHN NELSON, ***-**-****

Age: 50 Sex: MALE Race: WHITE

Weight: 287.6 lb [130.45 kg] (10/03/2023 09:16) Height: 71.5 in [181.6 cm] (10/03/2023 09:16)

Respiration: 18 (10/03/2023 09:16) Temperature: 97.6 F [36.4 C] (10/03/2023 09:16)

Pulse: 57 (10/03/2023 09:16) Blood Pressure: 134/72 (10/03/2023 09:16)
Pain Scale: 3 (09/19/2022 09:25)

DISCLAIMER: For patients being seen at DOD, Cerner Sites, and VHA facilities, Remote Medications, Remote allergies and adverse drug reactions (ADRs) may be incomplete. Patient medication list and allergies must be viewed using Joint Legacy Viewer (JLV). For patients seen at DOD, Cerner Sites and VHA facilities, I have reviewed patient medications using JLV.

ALLERGIES REMOTE AND LOCAL REVIEW:
FACILITY ALLERGY/ADR

No Remote Allergy/ADR Data available for this patient
AUDIE L. MURPHY MEMORIAL HOSP No Known Allergies

Remote and Local Allergies reviewed, and reactions confirmed.

Medications:

Active Inpatient, Outpatient and Clinic Medications (including Supplies):

Active Outpatient Medications	Status	Issue Date Last Fill	Refills	Expiration
1) IBUPROFEN 800MG TAB Qty: 200 for 67 days Sig: TAKE ONE TABLET BY MOUTH THREE TIMES DAILY WITH MEALS AS NEEDED FOR PAIN TAKE WITH FOOD OR MILK.	ACTIVE	Issu:10-03-23 Last:02-04-24	Refills: 1	Expr:10-03-24

Medication Reconciliation:

I have performed med rec with the patient/caregiver utilizing the Essential Medication List for Review (EMLR) MRT1. This included, under MRR1 active and pending prescriptions dispensed from local and remote VA prescriptions, DoD prescriptions, local non-VA medications, local inpatient medications orders, clinic medication orders and prescriptions that have expired or been discontinued in the past 90 days.

Active Medication List:

INCLUDED IN THIS LIST: Alphabetical list of active outpatient prescriptions dispensed from this VA (local) and dispensed from another VA or DoD facility (remote) as well as inpatient orders (local pending and active), local clinic medications, locally documented non-VA medications, and local prescriptions that have expired or been discontinued in the past 90 days.

Non-VA Meds Last Documented On: Jul 22, 2016

NOTE The display of VA prescriptions dispensed from another VA or DoD facility (remote) is limited to active outpatient prescription entries matched to National Drug File at the originating site and may not include some items such as investigational drugs, compounds, etc.

NOT INCLUDED IN THIS LIST: Medications self-entered by the patient into personal health records (i.e. My HealtheVet) are NOT included in this list. Non-VA medications documented outside this VA, remote inpatient orders (regardless of status) and remote clinic medications are NOT included in this list. The patient and provider must always discuss medications the patient is taking, regardless of where the medication was

dispensed or obtained.

OUTPT IBUPROFEN 800MG TAB (Status = Active)

TAKE ONE TABLET BY MOUTH THREE TIMES DAILY WITH MEALS AS NEEDED
FOR PAIN TAKE WITH FOOD OR MILK.

Rx# 25735750 Last Released: 1/30/24 Qty/Days Supply: 200/67

Rx Expiration Date: 10/3/24 Refills Remaining: 1

Indication: FOR PAIN

SUPPLIES

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MEDICATION REVIEW

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No changes made to medications during this visit.

Problem List:

Active problems - Computerized Problem List is the source for the following:

1. Exposure to potentially hazardous substance (SCT 133261000119105)
2. Plantar fasciitis
3. Back pain
4. Foot pain
5. Finger pain
6. Obesity
7. History of malignant melanoma of the skin
8. Indigestion
9. Hyperglycemia
10. Tobacco use
11. Sleep apnea
12. Chronic post-traumatic stress disorder
13. Hip pain
14. History of male erectile disorder
15. Dysphagia
16. Hernia

Chief Complaint: Vet presents for 6 months f/u

INTERIM HISTORY:

- f/u labs
- No new problems/concerns
- would like Prev 20
- No colonoscopy

History of Present Illness:

This is a 50-year-old male Veteran who presents to Victoria outpatient clinic for continued care reports he continues to have chronic pain to his left shoulder and left hip, he is also concerned with this "spot" on his back- he has hx of melanoma. He would like to discuss options for weight loss (he reports his wife is doing great taking monjourno) He also states he recently hired a personal trainer to help get him in shape.

Veteran is pleasant and friendly. Since last PC visit veteran has not had any ED visits, hospitals stays, or surgery.

Service Connection: He reports enlisting into the Army in 1994- he was in the infantry- was deployed x4 to Afghanistan and Iraq. He retired in 2015

After leaving the service- he got his certification as a teacher- taught in alternative school until 2 years ago when he retired. He reports that lasted about 2 months- went back to school to get his real-estate license- and has been selling houses. He reports he still coaches, umpires and referees at school games.

Family history: He denies any pertinent family hx.

Past surgical history: melanoma removed from back, teeth extraction

Medical Hx: Pre-DM, HTN, Obesity, OSA

Current health status:

Tobacco use: none

Alcohol use: none

Illicit drugs: None

Review Of Systems:

Skin: Patient denies any rashes, itching or change in hair/nails.

HEENT: Patient denies any headaches, no change in vision, no hearing difficulties, or neck or throat pain.

Respiratory: Patient denies any shortness of breath, wheezing, or congestion.

Cardiac: Patient denies any swelling of lower extremities heart skipping or high blood pressure.

Gastrointestinal: Patient denies any change in appetite, Constipation, diarrhea, or abdominal pain. He reports continued weight gain

Urinary: Patient denies any difficulty with urination, pain or burning. Reports no incontinence

Musculoskeletal: Patient report chronic pain, stiffness, and decreased joint motion to his left hip and shoulder

Neurological: Patient denies any headaches, seizures, tremors, or numbness and tingling.

Psychiatric: Patient denies any tension/anxiety/depression, or suicidal ideations. Reports no sleeping problems.

PHYSICAL EXAMINATION:

Temp: 98.3 F [36.8 C] (04/01/2024 09:20)

P: 70 (04/01/2024 09:20)

R: 20 (04/01/2024 09:20)

B/P: 116/80 (04/01/2024 09:20)

Weight: 304.3 lb [138.03 kg] (04/01/2024 09:20)

Height: 71.5 in [181.6 cm] (10/03/2023 09:16)

BMI:41.9

General Exam:

No acute distress, AAOx4, appropriate for situation

HEENT: Normocephalic, atraumatic, EOM, no glasses, no Hearing Aides

Neck: Supple, trachea midline, No JVD or masses, No carotid bruits

Lymph nodes: No cervical adenopathy

Cardio: Regular rate, regular rhythm, S1 S2 noted no murmur

Respiratory: Bil BS clear upon auscultation, resp even and unlabored, able to speak full sentences.

Abdomen: Soft, nontender, nondistended, bowel sounds present x4 quad

Extremities: Equal upper/lower ext strength, No cyanosis, swelling or deformity

noted, limited ROM

Psych: Coherent and logical. Denies current SI/HI/AVH. Insight and judgement are fair

Neuro: No focal deficits, CNS 1-12 intact

Skin: No lesion, rashes, skin warm and dry to touch

LABS:

CBC:

Collection DT	Specimen	Test Name	Result	Units	Ref Range
03/29/2024 10:23	BLOOD	WBC	7.2	10.e3/uL	4.0 - 10.0
03/29/2024 10:23	BLOOD	RBC	4.86	10.e6/uL	4.5 - 5.9
03/29/2024 10:23	BLOOD	HGB	14.8	g/dL	13.5 - 17.5
03/29/2024 10:23	BLOOD	HCT	44.4	%	41.0 - 53.0
03/29/2024 10:23	BLOOD	MCV	91.4	fL	78.0 - 98.0
03/29/2024 10:23	BLOOD	MCH	30.4	pg	26.0 - 34.0
03/29/2024 10:23	BLOOD	MCHC	33.3	gm/dL	32.0 - 36.0
03/29/2024 10:23	BLOOD	PLTS	278	10.e3/uL	150 - 400

Chem 20:

Date	Test	Result	Units	Normal Range	Specimen
03/29/2024	CREATININE	1.0	mg/dL	.7 - 1.3	SERUM
03/29/2024	UREA NITROGEN	17	mg/dL	7 - 25	SERUM
03/29/2024	GLUCOSE	92	mg/dL	70 - 105	SERUM
03/29/2024	SODIUM	142	mmol/L	136 - 145	SERUM
03/29/2024	POTASSIUM	4.7	mmol/L	3.5 - 5.1	SERUM
03/29/2024	CHLORIDE	105	mmol/L	98 - 107	SERUM
03/29/2024	CALCIUM	9.8	mg/dl	8.6 - 10.3	SERUM
03/29/2024	PROTEIN,TOTAL	7.1	g/dL	6.4 - 8.9	SERUM
03/29/2024	ALBUMIN	4.5	g/dl	3.5 - 5.7	SERUM
03/29/2024	TOT. BILIRUBIN	0.5		0.3 - 1.0	SERUM
03/29/2024	ALKALINE PHOSP	58	U/L	34 - 104	SERUM
03/29/2024	AST	19	IU/L	13 - 39	SERUM
03/29/2024	ALT	25	IU/L	7 - 52	SERUM
03/29/2024	ANION GAP	9.0		6 - 15	SERUM
03/29/2024	CO2	28.0	mmol/L	21 - 31	SERUM
03/29/2024	ICTERUS	0	NEG		SERUM
03/29/2024	LIPEMIA	0	NEG		SERUM
03/29/2024	HEMOLYSIS	0	NEG		SERUM
03/29/2024	OSMOLALITY-CAL	284.00	mOsm/kg		SERUM
12/23/2021	zzEGFR	>60			PLASMA
03/29/2024	EGFR CKD EPI	92			SERUM

A1C:

Date	Test	Result	Units	Normal Range	Specimen
09/26/2023	HEMOGLOBIN A1C	5.7	%	4.0 - 6.0	BLOOD

PROSTATIC SPECIFIC ANTIGEN - NONE FOUND

LIPID PROFILE Coll. dat3/29/24 10:23 9/26/23 10:13 6/6/22 08:55

CHOL	189	179	167
TRIG	95	97	88
HDL	51	46	44
LDL CHOLESTEROL	119.00	114.00	105.00

VLDL CHOL 19.0 19.0 18.0
LDL DIRECT

ASSESSMENT/PLAN:

melanoma - posterior trunk; less than 0.5cm soft tissue mass palpated at site, Community Care Consult placed today for Dermatology- will await consult for further evaluation and recommendations.

hip pain - left; chronic, Community Care Consult placed today for L hip x-ray,- will await consult for further evaluation and recommendations. has been seen by Ortho in the past and done PT, reports episodic exacerbation. community Care Consult placed today for Rheumatology- will await consult for further evaluation and recommendations.

Shoulder pain- left; chronic, Community Care Consult placed today for L hip x-ray,- will await consult for further evaluation and recommendations. has been seen by Ortho in the past and done PT, reports episodic exacerbation. Offered lidocaine patches- reports "don't help- Ibuprofen helps"- Community Care Consult placed today for Rheumatology- will await consult for further evaluation and recommendations.

Obesity- offered move program- declined at this time. Discussed benefit of weight loss with Veteran- will initiate Metformin bid low dose. He has hired a personal trainer. Discussed healthy food options. Voiced understanding

Prediabetic - (Hga1c 5.9 June 2022)No A1c available today- pt with family h/o DM; will initiate Metformin bid low dose. discussed low carb diet; no/low impact CV exercise, and weight loss.

HTN - controlled; no added salt diet; avoid tobacco products; exercise tobacco use - dip; quit 6/2019

Musculoskeletal pain - BACK, HIPS, KNEES; diclofenac gel offered- declined; oral NSAIDs, and tens unit, Educated on stretching and hydration. Veteran with notify PC team if does not improve.

Anemia - acute; mild; iron supplement ordered. Educated on foods high in iron.

GERD - transient; treats as needed OTC medications

PTSD - denies SI/HI, declined referral to MH; Educated pt on resources available at the VA including same day PCMHI and crisis line.

Umbilical hernia- surgical intervention by general surgeon - umbilical hernia repair Dec. 2023.

OSA - stable - voices no complaints- will continue to monitor.

Health Maintenance

- CRC: Adults age 45-75. iFOBT ordered
- Cholesterol and Lipid Disorder: . TC 95
- HCV: Age 18-79. Neg 2018
- HIV: Ages 13-64. Neg 2013

Follow-Up

Follow-Up Appointment: 6 months with PCP or sooner as needed

Pre-Clinic labs: CBC, CMP, A1c, Lipid, Vit D, Vit B12, TSH

CLINICAL REMINDER ACTIVITY**V17 ALL Weight Management-MOVE!:****VISN 17 MOVE! Clinical Reminder**

Patient's BMI (Body Mass Index) has identified patient as having a requirement to be evaluated for a weight management intervention.

Most recent weight: 304.3 lb [138.03 kg] (04/01/2024 09:20)

Most recent height: 71.5 in [181.6 cm] (10/03/2023 09:16)

Calculated BMI: 41.9

Provider Questions for Patient:

1. "What do you know about the benefits of weight loss?"

Veteran's Response:

2. "May I share information with you about the VA MOVE! weight management program?"

No

3. "Is it ok for me to refer you to a MOVE! program provider at this time?"

Weight Management Intervention:

Patient was offered referral to MOVE! Weight Management Program and declined due to:

Patient is NOT interested at this time. Pt provided MOVE! handout "So, You're Not Ready Yet!" - He reports he hired a personal trainer.

V17 P At Risk for Wandering Screen:

This patient does not meet "at-risk" for wandering criteria and does not have one or more of the following:

1. Is not legally committed; or
2. Does not have a court appointed legal guardian; or
3. Is not considered dangerous to self or others; or
4. Is not gravely disabled due to a mental disorder; or
5. Does not lack cognitive ability (either permanently or temporarily) to make relevant decisions; or
6. Does not have physical or mental impairments that increase their risk of harm to self or others (dementia, sedated, intoxicated, etc).

Avg Risk Colorectal Cancer Screen:

AVERAGE RISK colorectal cancer screening is due based on information available to this clinical reminder

Defer reminder for 3 months

Reason: pt reports ifobt was negative

/es/ THERESA O YBARRA

FNP-C Victoria Outpatient Clinic

Signed: 04/01/2024 11:03

Department of Veterans Affairs		REQUEST FOR SERVICES (RFS) FORM	
PREVIOUS AUTHORIZATION NUMBER: TODAY'S DATE (MM/DD/YYYY):		NOTE: The Request for Services (RFS) Form 10-10172 must be submitted via an approved method (HSRM, Electronic fax, Direct Messaging, traditional fax, or mail). Completion of this form is REQUIRED and MUST BE SIGNED by the requesting provider for further care to be rendered to a Veteran patient.	
SECTION I: VETERAN INFORMATION			
1. VETERAN'S LEGAL FULL NAME (First, MI, Last):		2. DOB (MM/DD/YYYY):	
3. VA FACILITY: STVHCS/Pink Team - Team Fax: (210) 443-0301		4. VA LOCATION: San Antonio, TX Ph: (210) 949-3850	
SECTION II: ORDERING PROVIDER INFORMATION			
5. REQUESTING PROVIDER'S NAME:		6. NPI #:	7. SPECIALTY:
8. OFFICE NAME & ADDRESS:			
9. SECURE EMAIL ADDRESS:			
10. PHONE NUMBER:	11. FAX NUMBER:		☐ 12. INDIAN HEALTH SERVICES (IHS) PROVIDER?
SECTION III: TYPE OF CARE REQUEST			
13. PLEASE INDICATE CLINICAL URGENCY (Urgent care is only applicable for requests that require less than 3 days to process. If care is needed within 48 hours or if Veteran is at risk for Suicide/Homicide, please call the VA directly on the same day as completed RFS form submission. Do NOT mark urgent for administrative urgency): ☐ ROUTINE ☐ URGENT			
14. DIAGNOSIS (ICD-10 Code/Description):		15. DATE OF SERVICE (MM/DD/YYYY) &/OR ANTICIPATED LENGTH OF CARE:	
16. CPT/HCPCS CODE &/OR DESCRIPTION OF REQUESTED SERVICES (Include units/visits, add second list page, if needed):			
17. HOW MANY VISITS HAVE OCCURRED SO FAR? (If known)		18. IS THIS A REFERRAL TO ANOTHER SPECIALTY? ☐ YES (If "YES," please fill out the Servicing Provider/Specialty information below) ☐ NO	
19. SERVICING PROVIDER'S NAME:		20. NPI #:	21. SPECIALTY:
22. OFFICE NAME & ADDRESS:			
23. SECURE EMAIL ADDRESS:			
24. PHONE NUMBER:		25. FAX NUMBER:	
SECTION IV: TYPE OF SERVICE REQUESTED			
26. OUTPATIENT CARE: ☐ PT ☐ OT ☐ SPEECH THERAPY		27. SURGICAL PROCEDURE: ☐ INPATIENT ☐ OUTPATIENT	
FREQUENCY & DURATION:		FACILITY NAME:	
28. ☐ IN-OFFICE PROCEDURE		29. INPATIENT CARE: ☐ LTACH ☐ ACUTE REHAB ☐ BH	
30. ☐ ADDITIONAL OFFICE VISITS (List # needed):		31. ☐ EXTENSION OF VALIDITY DATES	
32. ☐ EMERGENCY ROOM CARE		33. ☐ LABS (If done outside of office, please provide facility above)	
34. ☐ RADIOLOGY/IMAGING (If done outside of office, please provide facility above)		35. ☐ PRE-OPS LABS ☐ CHEST XRAY ☐ EKG ☐ OTHER:	
36. JUSTIFICATION FOR REQUEST (To avoid delays in care, include appropriate documentation such as office notes, current treatment plans, clinical history, laboratory results, radiology results &/or medications to support the medical necessity of services requested).			

VETERAN'S LEGAL FULL NAME (First, MI, Last):		
SECTION V: GERIATRICS AND EXTENDED CARE SERVICES (If applicable)		
37. ☐ COMMUNITY ADULT DAY HEALTH CARE ☐ COMMUNITY NURSING HOME ☐ HOMEMAKER/HOME HEALTH AIDE ☐ HOME INFUSION ☐ HOSPICE/PALLIATIVE CARE ☐ RESPITE ☐ SKILLED HOME HEALTH CARE ☐ OTHER: _____		
FREQUENCY & DURATION: _____		
38. JUSTIFICATION FOR REQUEST (To avoid delays in care, include appropriate documentation such as office notes, current treatment plans, clinical history, laboratory results, radiology results &/or medications to support the medical necessity of services requested).		
SECTION VI: HOME OXYGEN INFORMATION (If applicable)		
39. PAO2 AT REST:	40. O2 SAT AT REST:	41. OXYGEN FLOW RATE:
42. EXTENT OF SUPPORT (Continuous, Intermittent, Specific Activity):		
43. OXYGEN EQUIPMENT (Stationary/Portable):		
44. DELIVERY SYSTEM (Cannula, Mask, Other):		
SECTION VII: DME & PROSTHETICS INFORMATION (If applicable)		
45. HCPS FOR THE ITEM(S) BEING PRESCRIBED:		
46. BRAND, MAKE, MODEL, PART NUMBERS:		
47. MEASUREMENTS:		
48. QUANTITY:	49. ICD-10:	50. PROVISIONAL DIAGNOSIS:
51. DELIVERY/PICKUP OPTIONS: ☐ DELIVER TO ORDERING PROVIDER'S ADDRESS ☐ VETERAN WILL PICKUP AT THE VA MEDICAL CENTER ☐ DELIVER TO COMMUNITY VENDOR FOR DELIVERY & SETUP FOR DME ☐ DELIVER TO VETERAN'S HOME		
SECTION VIII: DURABLE MEDICAL EQUIPMENT (DME) EDUCATION & TRAINING (If applicable)		
Please see DME Requirements/Pharmacy Requirements - Community Care (va.gov) for URGENT DME requests.		
NOTE: Failure to thoroughly complete the RFS for DME will result in delayed patient care & prevent the VA from DME fulfillment.		
52. BEFORE DME WILL BE ISSUED, EDUCATION, TRAINING, &/OR FITTING OF DME (as applicable for the specific DME being ordered) TO THE VETERAN MUST BE COMPLETE. PLEASE INDICATE WHETHER THE FOLLOWING HAS BEEN COMPLETED FOR THE VETERAN. NOTE: If not completed, DME will be mailed to requesting provider's address to coordinate an alternative time for proper instruction on DME use.		A. EDUCATION: ☐ YES ☐ NO B. TRAINING: ☐ YES ☐ NO ☐ N/A C. FITTING: ☐ YES ☐ NO ☐ N/A
53. JUSTIFICATION FOR REQUEST (To avoid delays in care, include appropriate documentation such as office notes, current treatment plans, clinical history, laboratory results, radiology results &/or medications to support the medical necessity of services requested).		

VETERAN'S LEGAL FULL NAME (First, MI, Last):	
SECTION IX: THERAPEUTIC FOOTWEAR ASSESSMENT INFORMATION (If applicable)	
54. FILL OUT THE INFORMATION BELOW (If applicable): ☐ LEFT FOOT ☐ RIGHT FOOT ☐ BILATERAL ☐ PREFABRICATED THERAPEUTIC FOOTWEAR ☐ CUSTOM THERAPEUTIC FOOTWEAR	NOTE: For prescription of therapeutic footwear due to disease pathology resulting in neuropathy or peripheral artery disease.
NOTE: For prescription of therapeutic footwear for severe or gross foot deformity which cannot be accommodated with conventional footwear.	55. CHECK APPROPRIATE DIABETIC/AMPUTATION RISK SCORE: ☐ RISK SCORE 2: PATIENT DEMONSTRATED SENSORY LOSS (<i>inability to perceive the Semmes-Weinstein 5.07 monofilament</i>), DIMINISHED CIRCULATION AS EVIDENCED BY ABSENT OR WEAKLY PALPABLE PULSES, FOOT DEFORMITY, OR MINOR FOOT INFECTION, & A DIAGNOSIS OF DIABETES. ☐ RISK SCORE 3: PATIENT DEMONSTRATED PERIPHERAL NEUROPATHY WITH SENSORY LOSS (<i>i.e., inability to perceive the Semmes-Weinstein 5.07 monofilament</i>), AND DIMINISHED CIRCULATION, AND FOOT DEFORMITY, OR MINOR FOOT INFECTION & A DIAGNOSIS OF DIABETES, OR ANY OF THE FOLLOWING BY ITSELF: (1) PRIOR ULCER, OSTEOMYELITIS OR HISTORY OF PRIOR AMPUTATION; (2) SEVERE PERIPHERAL VASCULAR DISEASE (PVD) (<i>intermittent claudication, dependent rubor with pallor on elevation, or critical limb ischemia manifested by rest pain, ulceration or gangrene</i>); (3) CHARCOT'S JOINT DISEASE WITH FOOT DEFORMITY; & (4) END STAGE RENAL DISEASE.
DESCRIBE FOOT DEFORMITY AND ADDITIONAL DETAILS: 	NOTE: Only patients who are experiencing medical conditions noted in the risk scores can be prescribed therapeutic/diabetic footwear.
*ATTESTATION: I do hereby attest that the forgoing information is true, accurate, & complete to the best of my knowledge & I understand that any falsification, omission, or concealment of material fact may subject me to administrative, civil, or criminal liability.	
I do hereby acknowledge that VA reserves the right to perform the requested service(s) if the following criteria are met: (1) The patient agrees to receive services from VA (2) Service(s) are available at VA facility & are able to be provided by the clinically indicated date (3) It is determined to be within the patients best interest. Upon completion of the requested service(s), VA will provide all resulting medical documentation to the ordering provider. If all criteria listed are not true & VA agrees the service(s) are clinically indicated, VA will provide a referral for services to be performed in the community.	
I do hereby attest that upon receipt of order/consult results, I will assume responsibility for reviewing said results, addressing significant findings, & providing continued care.	
56. REQUESTING PROVIDER SIGNATURE (Required):	57. TODAY'S DATE (MM/DD/YYYY):

To facilitate timely review of this request, the most recent office notes & plan of care must accompany this signed form.

For more information please visit: https://www.va.gov/COMMUNITYCARE/providers/Care_Coordination.asp.

VA Community Care Medical Policies describe standard VA health care benefit for services and procedures that community providers may recommend as necessary for a Veteran. Prior to providing care, providers should use the Community Care Medical Policies as a reference when determining if a Veteran meets VA clinical criteria. When additional services are requested, Community Care Medical Policies will be used to determine approval by a clinical reviewer. Community Care Medical Policies & supporting information can be found at: <https://www.va.gov/COMMUNITYCARE/providers/Medical-Policy.asp>